



Suites 4-7, 193 Minna Jane Drive
Charlottetown, PE C1E 0C4

 Info@redsandspm.com

 www.redsandspm.com

 +1 782-377-7813

Understanding Facet mediated lower back pain and treatment options: Patient Information Leaflet

This leaflet provides information about a common cause of lower back pain called Facet Mediated Pain. It explains what it is, its symptoms, and the treatment options available. Please discuss this information with your doctor to help you make informed decisions about your care.

What are Facet Joints?

Your spine is made up of a column of bones called vertebrae. Between each vertebra, at the back of the spine, are two small joints called facet joints.

- They guide and limit the movement of your spine, allowing you to bend and twist.
- Like other joints in your body (e.g., your knee), they have a cartilage lining and a capsule around them, and they can become painful due to wear and tear or injury.

What is Facet Mediated Pain?

Facet Mediated Pain (also known as Facet Joint Arthropathy) occurs when these small joints become inflamed, damaged, or worn down. This is a common cause of chronic lower back pain, especially in older adults, but it can happen at any age. It is often caused by:

- Osteoarthritis: Age-related wear and tear.
- Injury: Such as a car accident or a fall.
- Repetitive Stress: From work or sports.
- Poor Posture.

Common Symptoms

Pain from the facet joints has some typical characteristics:

- Location: A dull ache in the lower back, sometimes felt just off to one side of the spine.
- Referred Pain: The pain may radiate into the buttocks or the back of the thighs (but it usually does not go down to the foot like sciatica).
- Pain with Movement: It is often worse when you bend backwards, twist your spine, or stand for long periods.
- Typically such pain gets better with bending forwards.
- Occasionally pain might present as sharp pain on coughing or sneezing in the lower back.
- Stiffness: You may feel stiff, especially first thing in the morning or after resting.
- Tenderness: The area over the affected joint may be tender to the touch.

Diagnosis

Generally imaging modalities like CT scan and MRI can show degenerative and arthritic changes in the Facet Joints these do not help with identifying Facet Joints as a source of pain. A definitive diagnosis is only possible after having Medial Brach Blocks as described later in this document.

Treatment Options

Treatment usually follows a step-by-step approach, starting with the simplest options.

Step 1: Conservative (Non-Invasive) Treatments

These are always the first line of treatment and may be all you need.

- Physiotherapy: To strengthen your core and back muscles, improve flexibility, and correct posture.
- Medication: Over-the-counter anti-inflammatory drugs (like ibuprofen) or pain relievers (like paracetamol). Your doctor may also prescribe muscle relaxants.
- Activity Modification: Avoiding activities that make the pain worse.
- Heat/Ice Packs: Can help reduce muscle spasms and inflammation.

If these treatments do not provide enough relief, your doctor may recommend interventional pain procedures.

Step 2: Diagnostic Medial Branch Blocks

Before considering longer-term solutions, your doctor needs to confirm that your pain is definitely coming from the facet joints. This is done with a diagnostic medial branch block.

What is it?

A local anaesthetic (numbing medicine) is injected onto the small nerves (medial branches) that supply the painful facet joint.

What happens?

The procedure is done using X-ray guidance to ensure accuracy. It is quick and you will be awake. This is a diagnostic test. If your pain improves significantly for a few hours after the injection, it confirms that the facet joint is the source of your pain. This result helps guide further treatment.

Step 3: Therapeutic Intra-Articular Cortisone Injection

If the diagnostic block is positive, one treatment option is an injection directly into the facet joint itself.

What is it?

A mixture of a long-acting steroid (cortisone) and a local anaesthetic is injected directly into the joint space.

How does it work?

The powerful anti-inflammatory effect of the steroid reduces the swelling and irritation inside the joint, providing pain relief.

What to expect?

Relief can last from several weeks to many months. It can be repeated if effective, but there are limits to how often you can have steroid injections. Some patients find no pain relief or a very short duration of pain relief.

Step 4: Radiofrequency Ablation (RFA) – A separate detailed RFA patient information leaflet is available.

If you get good pain relief from a diagnostic medial branch block and a very short or temporary pain relief with cortisone, RFA may be an excellent option.

What is it?

A procedure that uses heat energy to gently "turn off" the pain signals from the medial branch nerves to the spinal cord and the brain.

How does it work?

Using X-ray guidance, a small needle is placed next to the nerve. A micro-electrode is then used to create a tiny, precise heat lesion that stops the nerve from transmitting pain signals.

What to expect:

- The procedure itself takes 30-60 minutes.
- The pain relief is not immediate; it can take 2-3 weeks to feel the full effect.
- The relief is long-lasting, typically providing pain reduction for 9 months to 2 years.
- The nerves can eventually grow back, at which point the procedure can be repeated if successful.

Important Note: RFA only treats the pain. It does not change the underlying arthritis. The joint itself is still arthritic, but your brain no longer receives the pain message from it.

Summary and Key Points

- Facet joint pain is a common and treatable cause of lower back pain.
- A diagnostic set of injections is crucial to confirm the diagnosis before proceeding to longer-term treatments.
- Cortisone injections can provide good medium-term relief by reducing inflammation.
- Radiofrequency Ablation (RFA) can provide long-term relief by disabling the nerves that carry the pain signal.

Always talk to your pain specialist or doctor about which option is best for you. They will discuss the potential benefits, risks, and success rates in detail.

Questions? Contact Your Care Team

Before/after procedure:

 782-377-7813

 Info@redsandspm.com

In an emergency

→ Call 911, or visit your local emergency department

Disclaimer

This leaflet is for informational purposes only and is not a substitute for professional medical advice. Always consult a qualified healthcare provider for diagnosis and treatment.